

Medicare/Medicaid Hospice Regulations

Comprehensive Compliance Reference Guide (42 CFR 418)

FEDERAL ELIGIBILITY CRITERIA (42 CFR 418.24)

Four Core Requirements:

1. Medicare Part A Enrollment

Patient must be enrolled in Medicare Part A (Hospital Insurance)

2. Physician Certification

Attending physician and hospice medical director must certify terminal illness

3. Prognosis Requirement

Life expectancy of 6 months or less if disease runs its normal course

4. Written Consent

Patient or representative must sign election statement choosing hospice

DISEASE-SPECIFIC CLINICAL GUIDELINES

Cancer (LCD L33393)

- Metastatic disease or locally advanced with poor prognosis
- Declining functional status (PPS < 70%)
- Patient declines further curative treatment

Heart Disease (NYHA Class IV)

- Symptoms at rest despite optimal medical management
- EF < 20% or recurrent hospitalizations (3+ in 12 months)
- Persistent hypotension, cardiac cachexia

Pulmonary Disease (End-Stage COPD)

- Disabling dyspnea at rest or with minimal exertion
- FEV1 < 30% predicted after bronchodilator
- Progressive weight loss, cor pulmonale

Dementia (FAST Stage 7+)

- Unable to ambulate, dress, bathe independently
- Limited speech (< 6 intelligible words/day)
- Recent aspiration pneumonia, UTI, sepsis, or pressure ulcers

OPTIMAL 4-STEP REFERRAL PROCESS

1. IDENTIFY

Use screening tools during care planning to identify appropriate patients early

2. DISCUSS

Conduct goals-of-care conversation with patient/family about comfort options

3. CERTIFY

Obtain physician certification of terminal prognosis (< 6 months)

4. ELECT

Patient signs election form; hospice admission within 24-48 hours

⚠️ COMPLIANCE TIP

Document all conversations about hospice in the medical record. Include patient/family responses and any barriers discussed. This protects both the facility and supports continuity of care.